

Major events timeline for Team participating to the SMH Cryptic-Challenge (C-CHA)

(Please refer to the linked documents and data files detailing the schematic information provided here)

September 26: a nursing home in Kelvin Grove, transfers two patients to the **Royal Brisbane and Women's hospital**. Patients are 79 and 86 years old. Both have pneumonia. The day after three more patients from the same nursing home develop influenza-like symptoms. One 76 years old is admitted to the hospital on 09/28 and two more patients (age unknown) are admitted to hospital on 09/29.

September 30: Queensland Health the nursing home cluster activates a rapid response team, implements strict isolation at the nursing home, and begins contact tracing for staff and residents.

September 30 to October 3: 9 more patients from the nursing home (all aged over 75) are hospitalized and isolated. On 9/30, one of the caregivers (51 years old) of the nursing home is hospitalized, with atypical pneumonia.

October 4: the government escalate the alert.

- Diagnostic & Epidemiological Assessment: labs conduct urgent testing to identify the pathogen, while epidemiologists analyze case links and potential sources.
- Healthcare System Readiness: Hospitals and aged care providers receive alerts to monitor and report similar cases, with RBWH preparing isolation beds if needed.
- Public Communication & Coordination: Queensland Health briefs government officials, prepares a public advisory, and coordinates with national health agencies for potential escalation.

News is reported in Newspapers ([see Courier_mail_October_4](#))

October 5:

Children Health Queensland Hospital notifies Queensland Health of the family cluster (4, 12 years old kids and the parent). The 12 years old kid has developed atypical pneumonia. A response team starts investigations and contact tracing. 100 contacts are traced and followed (mostly schoolmates of the 12 years old kid).

Nine more patients are reported admitted to hospital from the nursing home are hospitalized (aged more than 75) along with one more caregiver (51 years old)

The Children Health Queensland Hospital outbreak is reported in the media and potentially connected with the nursing home outbreak ([see ABC_news_October_5](#)).

October 6:

Queensland Chief Health Officer (CHO) convenes a meeting with Metro North HHS, Queensland Health, and CDNA (Communicable Diseases Network Australia).

The Australian Health Protection Principal Committee (AHPPC) is briefed.

The National IHR Focal Point (NFP) (which sits within the Office of Health Protection in the Australian Department of Health) directly notifies WHO's Western Pacific Regional Office (WPRO) in Manila.

October 7.

The first two deaths (patients from the nursing home) are reported. The patients had several comorbidities and were aged 82 and 84

The WHO issue a first official alert ([see WHO_alert_october_7](#))

October 10. One more death is reported for a patient aged 78 from the nursing home.

The Australian Government issue a Public Health Advisory ([see Australian_advisory_October_10](#))

October 13.

The Queensland Chief Health Officer (CHO) convenes the press to announce that in the last 5 days the hospital network of Brisbane Metro North area has been notified of 25 new cases with acute respiratory infection admitted into the following hospitals:

- St Andrew's War Memorial Hospital
- St Vincent's Private Hospital

- Royal Brisbane and Women's Hospital
- North-West Private Hospital
- Children Hospital

Cases appear not to be linked to existing cases from the two recorded outbreaks. Two cases are in small children of 1 and 3 years old, respectively. These cases appear to be severe. One additional death is recorded among the hospitalized patients of the nursing home.

October 14.

Seven of the previously suspected hospitalized cases were positive to Flu and dropped from the suspected list. 13 additional suspected cases are hospitalized on October 14.

One additional suspected hospitalization negative to the respiratory virus panel has been notified in Townsville, a coastal city in northeastern Queensland, Australia. The patient has been hospitalized in Townsville University Hospital and has a history of travel from Brisbane. A contact tracing unit in Townsville is identifying passengers on the flight manifest and performing contact tracing.

See: [Queensland_Chief_Health_October_14](#); [Queensland_Advisory_October_14](#); [Australian_Advisory_October_14](#))

October 16.

Genetic identification of virus X based on meta-sequencing of clinical specimens from Australia by the Doherty lab. Sequence made publicly available on Genbank on October 17.

The results are discussed with the Australian Government Department of Health and Aged Care, Queensland Health and the WHO.

The WHO provides a revised advisory (see [WHO_Alert_October17](#)).

October 20.

Total number of hospitalizations for respiratory infections negative to the respiratory panel notified by city:

- 75 Brisbane Metro area, Queensland
- 1 Townsville, Queensland
- 1 Rockhampton, Queensland
- 8 Sidney, New South Wales

About 10% are Acute respiratory infections with atypical pneumonia.

Suspected Deaths:

- 6 Brisbane Metro area, Queensland (two are pediatric deaths).
- 1 Townsville, Queensland

Evidence for sustained ongoing Human to Human transmission from several family and nosocomial transmission events.

The Doherty lab reports that all deaths are positive for virus X. Clinical samples from all hospitalized patients with respiratory symptoms who are negative for common viruses are to be sent to the Doherty lab for testing.

October 20

Identification of the Causative Agent of Emerging Respiratory Epidemic (SPOK-24)
(see [WHO_Brief_SPOK](#))

October 22

PCR primers for virus X are developed by the Doherty lab.

October 25

WHO issues the first sitrep (see [who_situation_report_1](#))
First Paper with clinical features and outcomes of the first 70 patients (see [Lancet_October_25](#))

October 29

Paper Detailing the early transmission dynamic of the Virus (see [NEJM_October_29](#))
WHO issues the second sitrep with weekly data – week 43- (see [who_situation_report_2](#)).
Correspondence summarizing early finding and timeline (see [SPOK-24_NEJM_correspondence_October_29](#))

November 4

WHO issues the third sitrep with weekly data – week 44- (see [who_situation_report_3](#)).

SPOK-24 cases are detected in several places in the world (cases with travel history from Australia)

November 12

WHO issues the second sitrep with weekly data – week 45- ([see who_situation_report_4](#)).

WHO issues the declaration of PHEIC ([see who_press_release_PHEIC](#))

Australian Government issues a new advisory ([see Australian_Advisory_Nov_12](#)).

November 13: Official Naming is determined.

- Virus Name: Spooky-like Virus 2024 (SPOK-24)

- Disease Name: Spooky-Associated Respiratory Syndrome 2024 (SPOKARS-24)

November 15:

- Australian Government starts to provide daily and weekly timelines of Hospitalizations (see

1. **AUSTRALIA_total_weekly_hospitalizations.csv;**
 2. **Top_4_locations_AUS_weekly_hospitalizations.csv**
 3. **AUSTRALIA_weekly_cases.csv**
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November 23:

- Vietnam health authorities report anomalous increase of hospitalizations the capital city. Several cases (non-travel related) are confirmed to be caused by SPOK-24.
- South Korea confirms three independent clusters of SPOK-24 in Seoul (no history of travel).

November 30th:

- Japan confirms 12 hospitalizations due to SPOK-24 across the country.
- The CDC- TGS program confirms the detection of the SPOK-24 virus in sample from Wastewater in aircraft and the swab program at major airport. Positive samples have been detected in Los Angeles and San Francisco. See:
CDC_TGS_detection.csv

December 7th:

- During the week the US reports

1. 4 Hospitalizations due to SPOK-24 In San Francisco (Alameda County)

2. 4 Hospitalizations due to SPOK-24 in Seattle (King County).

- Singapore reports several SPOK-24 autochthonous infections and about 12 hospitalizations.
- Thailand reports several SPOK-24 autochthonous infections and about 20 hospitalizations.

December 14th :

- Vietnam reports 15 cumulative deaths due to SPOK-24
- South Korea reports 8 cumulative deaths due to SPOK-24. The provided numbers might underestimate the number of deaths and due to the difficulties in testing and notifications in hospitals of both countries.
- The United States has revised its testing protocols by removing the requirement for travel history to affected regions when screening for SPOK-24. Many countries have followed suit, updating their national guidelines to broaden testing eligibility. The World Health Organization now recommends that all patients presenting with respiratory symptoms be tested for SPOK-24, regardless of travel history. However, widespread shortages in diagnostic capacity are being reported globally

December 25th:

- US reports hospitalization due to SPOK-24 in New York City.
- As testing capacity ramps up, cases of SPOK-24 are now being detected across multiple U.S. states. However, widespread shortages in diagnostic supplies and laboratory capacity are reported nationwide, creating significant delays in confirmation and surveillance. The overlap with seasonal respiratory viruses further complicates the situation, as a surge in influenza-like illness (ILI) cases makes it challenging to distinguish SPOK-24 from more common infections based on symptoms alone.
- Two hospitalizations due to SPOK -24 are reported in Paris, France.
France starts regular reporting of SPOK-24 cases, hospitalization and deaths.
- Hospitalization and SPOK-24 cases are confirmed in Manchester, London and Glasgow.

UK starts regular reporting of SPOK-24 cases, hospitalization and deaths.

January 1st

- Number of cases and hospitalizations in the US are increasing. The CDC confirms the fifth deaths due to SPOK-24.
- Several deaths are confirmed due to SPOK-24 Vietnam, Japan and South Korea.
- The overall number of cases in Australia seems to slow down.

January 7

- First paper providing the assessment of the SPOK-24 severity (see **SPOK-24_Severity_Analysis**)

January 15th

- Number of hospitalizations seems to have plateau in Australia.
- CDC publishes weekly cases, hospitalizations and deaths timeseries at the state level. Data goes back to Nov. 30th and are updated weekly (end of the week): see: **US_deaths_nov_feb.csv; US_hospitalizations_nov_feb.csv, US_cases_nov_feb.csv**)
- Several countries start to provide available regular update for the number of SPOK-24 deaths (generally by date of notification).
See: World_Weekly_confirmed_deaths.csv

January 18th

- Australia made available the weekly time series of fatalities by date of deaths at the National level (time window October 15th to December 28th). See:
Australia_deaths_from_November2024.csv